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No. 96-1793

In The

Supreme Court of the United States

October Term, 1997

CEDAR RAPIDS COMMUNITY SCHOOL DISTRICT,

Petitioner,

vs.

GARRET F., A Minor By His Mother
and Next Friend, CHARLENE F.,

Respondent.

On Writ of Certiorari to the
United States Court of Appeals for the Eighth Circuit

BRIEF FOR RESPONDENT

DOUGLAS R. OELSCHLAEGER
Counsel of Record
DIANE KUTZKO
SHUTTLEWORTH & INGERSOLL, P.C.
Attorneys for Respondent
500 Firstar Bank Building
P.O. Box 2107
Cedar Rapids, Iowa 52406
(319) 365-9461

148172

(800) 274-3321 • (800) 359-6859

COUNSEL PRESS

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QUESTIONS PRESENTED

(1) Whether a school district is required to pay for nursing services for a disabled student as a "related service" under the IDEA.

(2) Whether there should be a bright-line rule that the Act's exclusion of "medical services" means only those services provided by a physician.

(3) Whether a school district is required to pay for nursing services under the facts of this case applying a multifactor test.

(4) Whether a school district is required to pay for nursing services under the facts of this case applying Iowa law.

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47 Fed. Reg. 33,846	22
47 Fed. Reg. 49,871-49,872	22
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128 Cong. Rec. 27,508	22
129 Cong. Rec. 33,316 (1983) (Statement of Rep. Biaggi)	23
143 Cong. Rec. § 4373 (daily ed. May 13, 1997) (Statement of Sen. Murray)	25
143 Cong. Rec. § 4407 (daily ed. May 14, 1997)	25
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H.R. Rep. No. 296, 99th Cong. 1st Sess. (1985)	19
H.R. Res. 558, 97th Cong., 2d Sess.	22
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S. Rep. No. 168, 94th Cong., 1st Session 5 (1975) (reprinted in 1975 U.S.C.C. Ann. 1425)	37

Table of Cited Authorities

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Section 608 of the Education of the Handicapped Act Amendments of 1983	22
American Academy of Pediatrics, *Children With Health Impairments*, 86 Pediatrics, 636 (1990)	38
American Association for Respiratory Care, *A Study of Chronic Ventilator Patients in the Hospital*, 11 (1991)	36
Hearings on H.R. 5, The IDEA Improvement Act of 1997: Before the House Subcomm. on Early Childhood, Youth & Families of the Comm. on Economic & Educ. Opps., 105th Cong., 1st Sess. 46 (1997)	25
Richard Nelson, *et al.*, *A Survey of Special Health Care Procedures Required by Students in Iowa Schools* 2 (Feb. 1990)	36
Report of the Committee on Education and the Workforce, House of Representatives, May 13, 1997, p. 91	20
United States Congress, Office of Technology Assessment, *Technology-Dependent Children: Hospital v. Home Care: A Technical Memorandum* 20-21 (1987)	35, 36
U.S. Department of Education, *Nineteenth Annual Report to Congress on the Implementation of the Individuals with Disabilities Education Act*, II-23 (1997)	36

STATEMENT OF THE CASE

A. Procedural Overview

The Iowa Department of Education (the Department) ruled, on December 16, 1994, that the Cedar Rapids Community School District (the District) was required to reimburse Garret F. for health care costs his parents incurred on his behalf for the 1993-1994 school year. The Department specifically held that the services required were both "related services" under federal law and "supportive services" under Iowa law. Pet. App. 52a, 54a, 63a. Petitioner chose to pursue its appeal in federal rather than state court. The United States District Court for the Northern District of Iowa granted summary judgment in favor of Garret, holding that the services were not within the "medical services" exclusion of the IDEA and therefore were "related services" that the School District was required to provide. Pet. App. 8a-16a. The Court of Appeals for the Eighth Circuit affirmed the District Court. Pet. App. 1a-7a. The District subsequently filed a petition for writ of certiorari, which this Court granted.

B. Garret F.

Garret F. was an energetic, active child when, in 1987, he sustained catastrophic injuries while riding as a passenger on his father's motorcycle. While Garret's mental abilities were unaffected, his spinal cord was severed. Garret, as a result of the accident and at all times relevant, has been quadriplegic, ventilator dependent and uses a wheelchair. His physical impairment has continued to have no effect on his mind or his mental abilities and he is doing well in school. Without the modest care he requires, he would be excluded from public school without any opportunity to obtain a meaningful education. Ruling — Iowa Department of Education. Pet. App. 19a, 59a. Tr. at 87-88. He would be relegated to homebound instruction. *Id*; Tr. at 87-88; 596-97.

C. The Dispute Between Garret F. and the District

In the fall of 1988, Garret F. started kindergarten in the District and has been in school in the District since that date. From

kindergarten through the fourth grade, Garret's family provided a personal attendant. In kindergarten, Garret's aunt, Stephanie Madison, performed the services of personal attendant.[1] From first through fourth grades, from 1989-1993, an LPN performed the services. Tr. at 31, 637-38.

Beginning in the summer of 1988, the year Garret entered kindergarten, his parents asked the District to provide for these services. The Freys were orally advised in 1989 that if they did not provide for the nursing care at school at their expense, Garret would not be permitted to attend the District's schools. Tr. at 637-38, 659-61. No written response was ever made by the District to the request for services until 1993. Tr. at 94. In the Spring of 1993, Garret's mother, Charlene, insisted on an answer in writing and received a denial. Tr. at 94-95.

The District's position throughout has been that, if it assumed responsibility for the care provided, it would be required to use a registered nurse, as opposed to an aide or LPN.[2] Pet. App. 25a; Tr.

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1. Ms. Madison was 18 years old when the school year began and had no RN or LPN certification and no prior nursing experience. Tr. at 82-83. The training received by Stephanie Madison in preparation for the 1988-1989 school year was basically to participate in Garret's care initially and be with family members as they attended to his needs. App. at 13-14; Tr. at 175-76.

2. The Administrative Law Judge (ALJ) commented as follows:

> We have here a situation where it has been determined by the Board of Nursing that an RN must be at the school site to supervise the health care needs of Garret. No mention is made of the current situation with services of an LPN provided by the family with distant RN supervision or the same at home, or not having nursing services during part of the day or on weekends. Why is Garret's situation in school, if the school is responsible, different than when the school is not responsible or Garret is elsewhere?

Pet. App. 61a-62a.

at 15-17, 353-54. In this regard, one of the District nurses testified she would not be comfortable delegating the care to an LPN. Tr. at 453-54.

The District buttressed its position by requesting a declaratory ruling from the Iowa Board of Nursing. Tr. at 351; 433-34. The ruling "based on information provided by the petitioner" was based on a misstatement of the stability level of Garret F.'s health by the District. Exh. 125 at 448; Tr. at 75-76; Pet. App. 31a. One critical error concerned Garret's autonomic hyper-reflexia condition (respiratory distress), which was stable at the time the first opinion was sought from the Board. Tr. at 76; 44-45.

Despite subsequent direct authorization from the Iowa Board of Nursing (via a revised opinion that was sought by the District and received after the Eighth Circuit rendered its opinion) to allow Garret's care to be delegated to an LPN, the District continues to maintain that it would be obligated to provide an RN to provide for Garret's needs. *See* Lodging of Solicitor General.

Because of this refusal, the Frey family has, at its own expense, arranged for an LPN to attend the entire school day with Garret and perform the services he needs. Pet. App. 20a-21a, Tr. at 31. The Freys paid $18.50 per hour for the 1993-1994 school year and $22.00 per hour for 1994/1995 for the nursing services at school. Pet. App. 20a-21a; Tr. at 89-90. The nurse works seven hours per day at school. *Id.*

This situation has existed since Garret's entry into the school system. It has resulted in the exhaustion of medical insurance Garret had through his father's employment; exhaustion of personal funds; and exhaustion of a second health insurance policy. Tr. at 91-92.

D. What Garret F. Needs To Attend School

For the year 1993-1994, as he has had since his kindergarten year, Garret had a set of personal needs that were required to be met while he was at school in order for him to participate in and benefit from the education program. As discussed below, these

needs are primarily met by Garret's family and friends when he is not in school. These needs are:

1. catheterization for urine retention once a day;
2. suctioning of his tracheostomy on an as-needed basis, generally once a day;
3. assistance in positioning his body;
4. ambu-bagging during suctioning;
5. assistance with food and drink;
6. assistance to "troubleshoot" his ventilator;
7. observation of breathing to determine if air is going into the lungs;
8. monitoring for signs of autonomic hyper-reflexia.

Pet. App. 20a.

Garret would be unable to attend school at all without the assistance that his attendant gives him. Tr. at 87-88. If unable to attend, his current educational option would be home-based instruction. Pet. App. 59a. Garret likes school and, with the assistance described above to meet his physical needs, is doing very well. Tr. at 87-88; Exh. 71 (Tr. at 4).

E. None of the Required Services Need to be Performed By a Physician or an RN.

Garret F.'s physician, Dr. Reinertson, testified in the administrative hearing that none of the services that Garret F. needs are required to be performed by a physician. A nurse's aide or LPN is sufficient for all services. App. at 4-10; Tr. at 47-48. He further stated that contrary to the conclusion repeatedly stated by the District on Exh. 91, ("Individualized Healthcare Plan"), Garret did not require continuous monitoring and assessment. App. at 18-19; Tr. at 627-28; Exh. 13-14, 91 (Tr. at 4). Accordingly, the

stated reason for the District's decision that the person providing the services be an RN:

> requires continuous monitoring and assessment, highly technical and extensive nature of care, life threatening prospect of inadequate care, mastery of numerous competencies,

Exh. 91 at 374, was not supported by the medical testimony.

F. When Garret is Not in School, the Services are Provided Primarily by His Parents and Other Non-Health Care Professionals, Including Friends.

The Frey family provides the same services to Garret at night and over the weekends that his LPN provides at school. Tr. at 68. The Frey family has received basic training to perform them. Tr. at 68-69. Mr. and Mrs. Frey received training in suctioning the tracheostomy and catheterization by observation in the hospital. Tr. at 653.

On a normal school day, Garret returns home at 2:45 p.m. From 3:00 p.m. until 11:30 p.m. Garret and his mother are alone with no services being provided by anyone other than Mrs. Frey. Tr. at 654. They have dinner together; they may go to the grocery store together. *Id.* Garret watches television programs, works on his computer, does his homework, and entertains friends. *Id.* They go outside and take a walk. On occasion Garret's mother takes a nap. App. at 19; Tr. at 654. When Garret goes outside he often goes by himself. He will go to the neighbors' house across the street by himself and has even taken a walk in his wheelchair around the block by himself. App. at 20; Tr. at 655. All of these activities are conducted without the assistance of a nurse or other health professional. Tr. 653-55.

During the nighttime hours, from Monday through Friday, a nurse comes to Garret's home to provide services that primarily involve turning Garret in his bed every two hours. Tr. at 672.[3]

Garret visits his father every other weekend and occasionally during the week. Tr. at 640. Garret usually brings a friend with him for the weekend visit. Tr. at 640. During these weekends, Garret's father takes care of the health needs of his son. Tr. at 641.

In addition, Garret's classmates and friends perform several of the services at issue including suctioning, ambu-bagging (provision of a manual breath by squeezing an air bag attached to the tracheotomy tube), assistance with feeding and drinking, shutting off alarms on the ventilator, helping him use his wheelchair and assisting with the ventilator. App. at 10-11; Tr. at 84-85.

G. The Potential Cost to the District Has Been Exaggerated.

The District receives additional funding because of Garret's special education status. According to the District, Garret's "rating" of "3.6 FTE" generates an additional $8,500-$9,000 for the District. Tr. at 491-93; 513-514; Exh. 48 (Tr. at 4); Pet. App. 36a. The District also receives approximately $3,600 in normal or regular funding because of Garret's attendance, for a total revenue to the District of $12,700. Pet. App. 36a; Tr. at 514. The total funding received by the District for the 17,500 students (before extra money for special education) is $63,000,000 (17,500 x $3,600) for 1993-1994. Tr. at 513-14. *See also* Brief for Amicus Curiae, National School Board Association, at 18, 26 (18,000 students x $5,483 = total funding of $98,940,000).

The District does provide a teacher associate to assist with turning pages in books and organizing his desk. Tr. at 278-79;

3. The nursing services are provided by an LPN at home rather than a non-licensed person due to the requirements of the health insurance policy in effect at the time of trial, which required a licensed agency for reimbursement. App. at 12-13; Tr. at 168. In contrast, the health insurance policy in effect in 1988 did pay for services given by Stephanie Madison at home, even though she was not a licensed provider.

522. The teacher associate's compensation is $9,548. Tr. at 523. A "first-year" RN could be hired for $27,981.79. Pet. App. 27a; Exh. 136; Tr. at 523-25. The District stipulated that if an RN were hired, the teacher associate would not be necessary, since the RN can also do all the things the associate does. Tr. at 278-79, 424-26, 465. Tr. at 465. Thus, the "net" cost of an RN would be approximately $18,000 for a first year RN, more for a more experienced RN. Pet. App. 28a.

There are 2,200 special education students in the District. Tr. at 533. Each of those students receives additional funding which, when received, is put into a category within the District budget and then sub-grouped with other categories. Tr. at 533-34. The District does not limit the expenditure for any specific student to the extra special education funding the District receives for that student. Tr. at 539-40. There is no spending limit legally imposed upon a School District with regard to an individual student with disabilities. Tr. at 604.

The funding issue was not, according the District, a primary reason for the denial. Tr. at 567. The "only reason" for the District's refusal to pay for related services and health services is the District's interpretation of legal requirements. Tr. at 524.

When the special education budget is "over-spent," the District incurs what it refers to as a "deficit." Tr. at 588. Even in these instances, the special education budgetary obligations are met by the District. Tr. at 590. During the five years preceding trial, all special education budgetary overrides were met. Tr. at 590. As the ALJ found, the District can obtain funds from the District cash reserve or receive additional funds from the State of Iowa. Pet. App. 37a; Tr. at 588. Significantly, as the ALJ found, the District has identified and weighted Garret for funding under state law. Pet. App. at 58a.

H. The District Provides Most of the Needed Services to Other Children.

The District provides the following services to other children at its cost: (1) intermittent catherization (done primarily by teachers

or teacher assistants), Tr. at 286, 335; (2) assistance in consuming food and drinking water and other beverages (often by nonlicensed personnel), Tr. at 290, 382; (3) monitoring of blood sugar levels for diabetic students (which Garret does not need), Tr. at 455-57; (4) suctioning of tracheostomies, Tr. at 332-34; (5) tube feeding (which Garret does not require), Tr. at 336; and (6) positioning services. Tr. at 556.

I. The Department's Decision was Based on State as Well As Federal Law

An administrative hearing was held by the Iowa Department of Education. The Department ruled that under *both* state and federal law, the District was obligated to reimburse Garret's mother for his health care costs for the 1993-1994 school year and to continue to provide "school health care for Garret as a related service." Pet. App. 58a, 63a.

SUMMARY OF ARGUMENT

The United States Court of Appeals for the Eighth Circuit correctly held that Garret F. was entitled to nursing services as related services under the IDEA. The Court of Appeals' ruling was correct under the plain language of IDEA, the relevant regulations, and the Act's legislative history, all as interpreted by this Court in *Irving Independent School District v. Tatro*, 468 U.S. 883 (1984) and other cases. Despite the District's assertions to the contrary, the IDEA is not merely "about education." It is about *access* to education. It specifically provides that: "all children with disabilities have available . . . a free appropriate public education . . . and related services designed to meet their unique needs . . ." 20 U.S.C. § 1400(c) (1994). Without meaningful access, including the provision of a broad range of supportive services, the goal of a "free and adequate public education" would be a hollow promise. *See Tatro*, 468 U.S. at 891.

The importance of meaningful access is illustrated by the facts of this case. The services are simple, straight-forward acts that can be provided by a nurse's aide or an LPN, and, in fact, have been

provided by Garret's friends, family, and a number of nonlicensed providers. The services are not required to be performed by a physician. The services can be provided at no additional cost to the District by the existing educational aide, who is an LPN and could provide the services. Without this care, however, Garret is condemned to miss out on school and would be kept at home in a segregated, home-based program.

This Court's decision in *Tatro*, 468 U.S. 883 (1984), supports a "bright-line" rule for cases such as this one. The bright line is drawn between services that must be performed by physicians (except those performed for the purpose of diagnosis or evaluation) and those provided by other providers, including school nursing services. The bright-line test would afford recipients of funds under the IDEA clear notice of their obligation. *See Tatro*, 468 U.S. at 891.

The bright-line test is also consistent with the clear language of the statute and the regulations of the Department of Education. The Court of Appeals properly deferred to the Secretary of Education regulations, which interpret IDEA to include as "related services" the health services provided by a school nurse or other qualified nonphysician, but to exclude as "medical services" those services that must be provided by a physician, other than for diagnosis and evaluation. Deference to the federal agency's interpretation is proper. *Chevron U.S.A., Inc. v. Natural Resources Council, Inc.*, 467 U.S. 837 (1984).

As noted by this Court in *Tatro*, it could be inferred that the expense issue was taken into account by Congress in excluding "medical services" provided by physicians. 468 U.S. at 892. The line having been drawn, expense should not be a determining factor as to whether nursing services are excluded, since this will lead to protracted litigation in many cases. It should also be noted that the expense in this case has been exaggerated: the services can and have been provided by an aide or an LPN, not an RN, at no additional cost to the District and can be and have been provided by non-professionals, including family members and friends.

While "undue burden" with regard to cost is not a factor under the IDEA, even if it were taken into consideration, the District's cost

arguments fail on the record in this case. The additional cost is nil if an LPN is used. The cost is minimal if an RN is used compared to the resources of the District. The District also has funding options available to it to obtain additional funds if it chooses, including funding from the State of Iowa. Further, the record is devoid of any evidence that the services required are outside the competence of the District's school nursing personnel. These are rudimentary services, most of which the District already provides to other children.

The District's proposed multifactor analysis is unworkable and, if adopted, would result in costly administration and litigation in future cases. However, even if this Court were to adopt a multifactor test in this case, Garret would be entitled to reimbursement for the requested nursing services due to the facts of this case.

Finally, the judgment should be affirmed because the services are required to be provided under state law.

In summary, the Eighth Circuit properly held that the services are required related services. Further, deference to rulings of state educational authorities, such as that of the Iowa Department of Education, is an essential part of implementation of the IDEA. *Board of Educ. of Hendrick Hudson Cent. Sch. Dist. v. Rowley*, 458 U.S. 176, 204-07 (1982). The obligation of the District exists under both federal and state law. For these reasons the judgment should be affirmed based on application of the bright-line test, or application of a multifactor test, or on state law grounds.

ARGUMENT

I.

THE IDEA REQUIRES THE DISTRICT TO PROVIDE THE REQUESTED NURSING SERVICES TO GARRET F.

Congress' purpose in enacting the Individuals with Disabilities Education Act (IDEA) (and its predecessor the Education of the Handicapped Act) was to "make public education available to handicapped children" *and* "to make such access meaningful." *Board of Educ. of Hendrick Hudson Cent. Sch. Dist. v. Rowley*, 458 U.S. 176, 192 (1982). The Act is therefore not merely "about education" as the District asserts (Petitioner's Brief at 12) but about providing access, which often includes the provision of needed health-related services. As this Court elaborated in *Irving Independent School District v. Tatro*, 468 U.S. 883 (1984), consistent with the goal of access to a "free and appropriate public education," the Act requires a two-part analysis: first whether a requested service is a "supportive service . . . required to assist a handicapped child to benefit from special education;" and second whether the service is excluded from the definition as a "medical service" (other than those required for diagnosis and evaluation). *Id.* at 886.

Despite the District's attempts to paint the services needed by Garret as "intensive, one-on-one full-time nursing care that Garret needs to stay alive" (Petitioner's Brief at 13), the record is overwhelmingly to the contrary. Tr. at 43-53. All of the services required by Garret can be performed by a nursing assistant or aide or a licensed practical nurse. *Id.*; Exhs. 26, 78 (Tr. at 4). Neither a registered nurse nor a physician is required. Indeed, the care the District persists in labeling "intensive, one-on-one full-time nursing care" is provided by a family member or friends when Garret is not at school. App. at 10-11; 19-20; Tr. at 32-34; 84-85; 641-41; 654-55. Without these services, Garret would not have "meaningful access" to an education. Tr. 87-88. He would be consigned to homebound instruction and be deprived of the stimulation provided by a classroom. Pet. App. 59a; Tr. at 87-88.

The language of the IDEA, the related regulations, the Act's legislative history and this Court's prior decisions interpreting the IDEA all support the determination by the ALJ that the nursing services required by Garret are "related services" that the District is required to provide at its cost in order to afford Garret meaningful access to a free appropriate public education.

A. The Language of the Statute and Regulations as Interpreted by the Secretary of Education Supports a Bright-line Test and Provision of the Requested Services in This Case.

Neither the plain language of the statute nor its purpose, 20 U.S.C. § 1400(c), supports the District's argument that Garret's needs are "medical services." Nowhere in the statutory language are the services required by Garret F. defined as excluded "medical services." On the contrary, the use of the broadly inclusive term "supportive services" in the enumeration of "related services" supports Garret F.'s position that these services must be provided.

The IDEA provides for a "free appropriate public education," which emphasizes special education and related services designed to meet a disabled student's unique needs. 20 U.S.C. § 1400(c). Such an education is defined as "special education and related services," which have been provided at public expense, under public supervision and direction, and without charge, and which are provided in conformity with the individualized education program required under the statute. 20 U.S.C. § 1401(a)(18).

The key statutory provision in this case is 20 U.S.C. § 1401(a)(17):

> The term "related services" means transportation, and such developmental, corrective, and other supportive services (including speech pathology and audiology, psychological services, physical and occupational therapy, recreation, including therapeutic recreation, social work services, counseling services, including rehabilitation counseling services, and medical services, except that such medical services shall be for

> diagnostic and evaluation purposes only) as may be required to assist a child with a disability to benefit from special education, and includes the early identification and assessment of disabling conditions in children.

The implementation of the IDEA is provided for in 20 U.S.C. § 1402(a), as amended. This section provides for the Office of Special Education Programs "which shall be the principal agency in the Department for administering and carrying out this Act [20 U.S.C. §§ 1400 *et seq.*] and other programs and activities concerning the education and training of the handicapped." 20 U.S.C. § 1402(a) (1991).

Under the regulations promulgated by the Department of Education, a "related service" is defined as any service "required to assist a child with a disability to benefit from special education." 34 C.F.R. § 300.16. A related service expressly includes "school health services." *Id.* "School health services" means "services provided by a qualified school nurse or other qualified person." 34 C.F.R. § 300.16(b)(11).

Medical services are expressly included as a related service but are limited in definition to "services provided by a licensed physician to determine a child's medically related disability." 34 C.F.R. § 300.16(b)(4). Related services, therefore, include a physician's diagnostic and evaluation services, but not treatment services. 20 U.S.C. § 1401(a)(17).

Under the Department of Education regulations, medical services are defined as "services provided by a licensed physician." 34 C.F.R. § 300.16(b)(4). The services Garret F. requires are not "medical services" and must be provided by the District as "related services."[4]

4. In the alternative, these services also appear to be "supplementary aids and services" necessary to "assure that, to the maximum extent appropriate, children with disabilities . . . are educated with children who (Cont'd)

B. This Court's Approach In *Tatro* Supports a Bright-line Test; The Test is Also Consistent With the Court's Decision in *Rowley*.

This Court first addressed the issue of whether nursing services fall within the scope of covered "related services" under the IDEA and the Department of Education regulations in *Tatro*, 468 U.S. 883 (1984). *Tatro* involved the issue of whether clean intermittent catheterization (CIC), a procedure involving the insertion of a catheter into the urethra to drain the bladder, was a related service under the IDEA. 468 U.S. at 885.

In *Tatro* this Court stated

> the regulations actually define only those "medical services" that are owed to handicapped children: services provided by a licensed physician to determine a child's medically related handicapping condition . . . *presumably this means that "medical services" not owed under the statute are those 'services by a licensed physician' that serve other purposes.*

468 U.S. at 892 n.10 (emphasis added). The Court found that:

> this definition of "medical services" is a reasonable interpretation of congressional intent. Although Congress devoted little discussion to the "medical services" exclusion, the Secretary could reasonably have concluded that it was designed to spare schools from an obligation to provide a service that might well prove unduly expensive and beyond the range of their competence.[5] From this understanding of congressional purpose, the Secretary could reasonably have concluded that Congress intended to impose the obligation to provide school nursing services.

468 U.S. at 892-93.

The Court further stated that Congress plainly required schools to hire various specially trained personnel to help handicapped children and that school nurses have long been a part of the educational system. The Court determined that the "Secretary could therefore reasonably conclude that school nursing services are not the sort of burden that Congress intended to exclude as a 'medical service.' " The Court reasoned that "[b]y limiting the 'medical services' exclusion to the services of a physician or a hospital, both far more expensive, the Secretary has given a permissible construction to the provision." 468 U.S. at 893-94. Deference to the agency interpretation is proper. *Chevron U.S.A. Inc. v. Natural Resources Defense Counsel, Inc.*, 467 U.S. 837 (1984).

The Court then noted several limitations that "should minimize the burden [of costs] the Petitioner fears":

> First, to be entitled to related services, a child must be handicapped so as to require special education . . . Second, only those services necessary to aid a handicapped child to benefit from special education must be provided, regardless how easily a school nurse or a lay person could furnish them . . .Third,

(Cont'd)
with children who are not disabled," 20 U.S.C. § 1412(5)(B). This Court noted the availability of this alternative in *Tatro*, 468 U.S. at 895 n.13. Congress defined the phrase "supplementary aids and services" in the 1997 IDEA Amendments (it was previously not defined in the statute or regulations) to mean "aids, services, and other supports that are provided in regular education classes . . . to enable children with disabilities to be educated with nondisabled children to the maximum extent appropriate." Pub. L. No. 105-17, Title I, § 101, 111 Stat. 37 (1997) (to be codified at 20 U.S.C. § 1401(29)).

5. The Petitioner omits any discussions of the second half of this language, i.e., "and beyond the range of their competence." There is no dispute in this record that the services are within the range of the competence of the providers the District has traditionally employed. In fact, with limited exceptions related to the ventilator and autonomic hyperflexia, the District has provided all of the services to other children within the District. *See, supra* Section H — Statement of the Case. The District's Amicus also lists school nurse services at pages 19-21 of its brief which include several of the services needed here.

> the regulations state that school nursing services must be provided only if they can be performed by a nurse or other qualified person, not if they must be performed by a physician.

468 U.S. at 894.

The Court in *Tatro* did *not* conclude that cost was a factor to be considered.[6] It merely observed that Congress and the Department of Education may have used expense, as well as limitations of educators' expertise in medical matters, for making a distinction between nursing services and physician treatment in determining what constitutes a related service. 468 U.S. at 892-93.

Tatro recognized an important principle. A service that enables a handicapped child to remain at school during the day is an important means of providing the child with the meaningful access to education that Congress envisioned: "The Act makes specific provision for services, like transportation, for example, that do no more than enable a child to be physically present in class. . ." 468 U.S. at 891. *Tatro* affirms the principle that health services do the same and must therefore be provided. *Id.*

The result urged here is also consistent with the Court's decision in *Board of Educ. of Hendrick Hudson Cent. Sch. Dist. v. Rowley*, 458 U.S. 176 (1982) which first addressed the predecessor statute to the IDEA. As a threshold matter, *Rowley*'s extended discussion and analysis of the Act's legislative history, discussed *infra* at I(C), demonstrate that the intent of Congress was to make available the services needed by children such as Garret F. unless performed by a physician. Further, *Rowley*, in contrast to this case, did not address the issue of access to a free appropriate public education. In *Rowley*, there were findings that the student was receiving an "adequate education," including personalized

6. Indeed, the IDEA does not contain an "undue burden" defense, as does the Americans With Disabilities Act. 42 U.S.C. § 12112(b)(5)(A).

education. 458 U.S. at 210. It was therefore uncontested that the student, who was deaf, had access to a free public education, *i.e.*, that the District was providing sufficient services at its cost to provide access to a free appropriate education under the Act. The dispute centered around whether the District was required to assume the cost of a sign language interpreter. 458 U.S. at 184-85. The issue in *Rowley* was therefore the level of services that the District was obligated to provide at its cost. 458 US. at 200. The facts of this case are starkly different; it is uncontested that without the requested nursing services, Garret F. would not be able to attend school and would be consigned to homebound instruction.

The services Garret F. needs are accordingly "related services" not "medical services," and the District is required to provide for them. The Eighth Circuit properly followed this Court's prior decisions, including *Tatro*, which correctly interpreted the IDEA and the regulations of the Secretary of Education.

C. The Legislative History of the IDEA Supports the Provision of the Requested Health Services to Garret F.

The legislative history of the IDEA also supports the provision of nursing services at the District's cost. In *Board of Education v. Rowley*, 458 U.S. 176 (1982), this Court first addressed the IDEA and the history of educating children with disabilities:

> Dissatisfied with the progress being made under [the Elementary and Secondary Education Act of 1965 and the Education of the Handicapped Act (L. 91-230)] and spurred by two District court decisions holding that handicapped children should be given access to public education, Congress in 1974 greatly increased federal funding for education of the handicapped and for the first time required recipient states to adopt "a goal of providing full educational opportunities to all handicapped children." Pub. L. 93-380, 88 Stat. 579, 583 (1974 Statute). The 1974 statute was recognized as an interim measure only, adopted "in

> order to give the Congress an additional year in which to study what if any additional federal assistance [was] required to enable the States to meet the needs of handicapped children." HR Rep, at 4. The ensuing year of study produced the Education for All Handicapped Children Act of 1975.

458 U.S. at 179-80 (citations omitted).

The Court noted that portions of the statute reflected on congressional intent:

> Congress found that of the roughly eight million handicapped children in the United States at the time of the enactment, one million were "excluded entirely from the public school system" and more than half were receiving an inappropriate education.

458 U.S. at 189. Prior to the Act "many of these children were excluded completely from any form of public education or were left to fend for themselves in classrooms designed for education of their nonhandicapped peers." *Id.* The Court noted the

> House Report begins by emphasizing this exclusion and misplacement, noting that millions of handicapped children were either totally excluded from schools or [were] sitting idly in regular classrooms awaiting the time when they were old enough to "drop out".

Id.

One of the Act's two principal sponsors in the Senate urged its passage in similar terms:

> While much progress has been made in the last few years, we can take no solace in that progress until all handicapped children are, in fact receiving an education. *The most recent statistics provided by the Bureau of Education for the Handicapped estimate that . . . 1.75 million handicapped children did not*

> *receive any educational services, and 2.5 million handicapped children are not receiving an appropriate education.*
>
> * * *
>
> Congress sought "to provide assistance to the States in carrying out their responsibilities under . . . the Constitution of the United States to provide equal protection of the laws."

458 U.S. at 191, 198 (emphasis added).

Petitioner and its Amicus, National School Boards Association, make much of the argument that spending for special education has increased since the late 60s. This argument misses the point, which is that children with disabilities are finally getting the educational opportunity of which they have been so deserving for so many years. *See e.g.* S. Rep. 91-634 at 146. The argument also ignores the financial support Congress does supply, 20 U.S.C. § 1411; 34 C.F.R. § 300.184 (1996), which is available to the District under State law.

In 1991, the IDEA was renamed and amended. The legislative history indicates that a principal purpose of the 1991 IDEA was to strengthen and coordinate pre-existing federal programs and requirements for children and youth with disabilities. H.R. Rep. No. 102-198 at 1-2 (1991). Significantly, in 1991, seven years after this Court's decision in *Tatro*, Congress could have remedied the effect of the distinction made in *Tatro* between excluded medical services (those provided by a physician) and included services (those provided by other providers). It did not do so.

While Congress in the past has chosen to overrule legislatively a decision of the Court interpreting the IDEA, *Smith v. Robinson*, 468 U.S. 992 (1984)[7], it did not so act when it reauthorized the

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7. *See Mrs. W. v. Tirozzi*, 832 F.2d 748, 754-55 (2nd Cir. 1987); H.R. Rep. No. 296, 99th Cong. 1st Sess. 4-6 (1985).

IDEA in 1997. *See* 20 U.S.C. § 1412(5)(A) (1997); *see also*, Report of the Committee on Education and the Workforce House of Representatives, May 13, 1997, p. 91.

In light of the legislative history of the IDEA, there can be no doubt that the services Garret needs must be provided by the District, and the decisions of the Iowa Department of Education, the District Court and the United States Court of Appeals for the Eighth Circuit should be affirmed. *See S-1 v. Turlington*, 635 F.2d 342, 347 (5th Cir. 1981), *cert. denied*, 454 U.S. 1030 (The IDEA must "be broadly applied and liberally construed in favor of providing a free and appropriate education to handicapped students").

The District's legislative history arguments are not well-founded. First, Petitioner's selective quotation of the recent amendments to the IDEA, Pub. Law 105-17, is significant. Petitioner, in its efforts to argue that the IDEA is limited to educational services, omitted the following congressional findings related to disability. These findings underscore the importance to Congress of *access* to education:

> (c) Findings — The Congress Finds the Following:
>
> (1) Disability is a natural part of the human experience and in no way diminishes the right of individuals to participate in or contribute to society. Improving educational results for children with disabilities is an essential element of our national policy of ensuring equality of opportunity, full participation, independent living, and economic self-sufficiency for individuals with disabilities.
>
> * * *
>
> 5) Over 20 years of research and experience has demonstrated that the education of children with disabilities can be made more effective by —

> (a) having high expectations for such children and ensuring their access in the general curriculum to the maximum extent possible;
>
> * * *
>
> (c) coordinating this Act with other local, educational service agency, State, and Federal school improvement efforts in order to ensure that such children benefit from such efforts and that special education can become a service for such children rather than a place where they are sent;
>
> (d) providing appropriate special education and related services and aids and supports in the regular classroom to such children whenever appropriate[.]

Second, the District further argues that because Congress did not narrow or "clarify" the "medical services" exclusion in 1990, this confirms that Congress did not want the Act to become more than an education law. Petitioner's Brief at 18. The District claims that Congress should have responded to the *Detsel* and *Bevin H.* decisions and clarified the law. However, the fact that it did not do this could, and more logically should, be interpreted to mean that Congress was satisfied with this Court's ruling in *Tatro*, which gave deference to the Agency's regulations. This applies with equal force to the District's argument that the definition of medical services should have been clarified in June of 1997. Petitioner's Brief at 19.

Further, Congress' actions in response to early proposed regulations confirm that the Secretary's "related services" regulation is consistent with the statute. On August 4, 1982, the Secretary of Education issued proposed regulations that would have changed various aspects of the special education regulations and, specifically, would have narrowed the class of services that must be provided under IDEA. The proposal would have eliminated "school health services" from the definition of "related services;" it would have

expanded the "medical services" exclusion (which the Secretary noted meant "services provided by a licensed physician," 47 Fed. Reg. 33,838) to mean "services relating to the practice of medicine"; and it would have allowed school districts to place "reasonable limitations" on provision of related services, including limitations based on the level, frequency, location, and duration of the services, and the qualifications of the service providers. *See id.* at 33,838-33,839; 33,846.

The proposed regulations provoked public and Congressional opposition. Congressional criticism specifically targeted a few of the proposed amendments, one of which was the amendment to the "related services" regulation. *See* 128 Cong. Rec. 21,793 (1982) (Statement of Rep. Bonker) (condemning proposed "changes that would permit limitations in necessary health services for handicapped children"). On September 10, 1982, Congress enacted § 305(b) of Pub. L. No. 97-257, 96 Stat. 874, expressing the sense of Congress that the proposed regulations should not become effective and should not be transmitted to Congress while it was not in session. *See also* H.R. Res. 558, 97th Cong., 2d Sess. 2-3 (reported out of House Committee on Education and Labor on September 30, 1982). On November 3, 1982, the Secretary withdrew six of the most controversial changes, one of those being the proposal to narrow the "related services" regulation. *See* 47 Fed. Reg. 49,871-49,872; *see also* 128 Cong. Rec. 27,508 (1982).

The following year, Congress enacted Section 608 of the Education of the Handicapped Act Amendments of 1983, which states that the Secretary

> may not implement . . . any regulation prescribed pursuant to this Act which would procedurally or substantively lessen the protections provided to children with disabilities under this Act, as embodied in regulations in effect on July 20, 1983 (*particularly as such protections relate to . . . related services . . .*), except to the extent that such regulations reflect the clear and unequivocal intent of the Congress in legislation.

Pub. L. No. 98-199, Title VI, § 608(b), 97 Stat. 1359, (codified as 20 U.S.C. § 1407(b)) (emphasis added). That provision was enacted in direct response to the changes proposed and then withdrawn by the Secretary in 1982. *See* 129 Cong. Rec. 33,316 (1983) (Statement of Rep. Biaggi). Under the circumstances, Section 1407(b) can only be seen as congressional confirmation that the current "related services" regulation reasonably interprets congressional intent, as well as a rejection of a multifactor determination of "related services" that would take into account the extent or nature of the services in each case.

> Where, as here, "Congress has not just kept its silence by refusing to overturn the administrative construction, but has ratified it with positive legislation," we cannot but deem that construction virtually conclusive.

Commodity Futures Trading Comm'n v. Schor, 478 U.S. 833, 846 (1986). The record of Congress' actions clearly refutes the District's legislative history argument.

D. Petitioner's Statutory Construction Arguments Must Fail Because They Are Contrary to the Intent of Congress.

Under well-established principles of statutory construction, while the "starting point in every case involving construction of a statute is the language itself," *Ernst & Ernst v. Hochfelder*, 425 U.S. 185, 197 (1976), all provisions of a statute must be harmonized and must be read so as to be consistent with other expressions of legislative intent. *Kelly v. Robinson*, 479 U.S. 36, 43 (1986). In construing a statute the Court will not be "guided by a single sentence or member of a sentence, but [instead] look[s] to the provisions of the whole law, and to object and policy." *Offshore Logistics, Inc. v. Tallentire*, 477 U.S. 207, 222 (1986) (citations omitted). Consistent with these principles, the District's statutory construction arguments must fail.

The analysis at pages 14 and 15 of Petitioner's Brief concerning the meaning of the term "medical services" of Petitioner's Brief simply misses the point. Petitioner quickly moves from the statutory language of the IDEA and focuses not on "medical services" as defined in the statute and regulations but instead couples the word "medical" with a variety of other words such as "care," "assistance," "devices," "benefits," and "providers." However, a careful analysis of the United States Code reflects that the term "medical services" generally if not universally means services provided by a physician. For example, in 5 U.S.C. § 8103(a)(3) employees are given the right to "initially select a physician to provide medical services, appliances, and supplies . . ." In 10 U.S.C. § 1588(a)(1) the Secretary is authorized to accept "voluntary medical services, dental services, nursing services, or other health-care related services." In 12 U.S.C. § 1715(w)(a)(1) the terms "skilled nursing care" are used in the same sentence as "and related medical services." 26 U.S.C.A. § 9712(c)(3)(F) provides for "selecting the most efficient physicians and state-of-the-art utilization management techniques, including ambulatory care techniques, for medical services delivered by the managed care network."

In 38 U.S.C. § 101(28)

> the term "nursing home care" means the accommodation of convalescences or other persons who are not acutely ill and not in need of hospital care, but who require nursing care and related medical services, if such nursing care and medical services are prescribed by, or performed under the general direction of, persons duly licensed to provide such care.

42 U.S.C. § 280(a)(1) provides

> to identify low-income individuals who can avoid institutionalization or prolonged hospitalization if skilled medical services, skilled nursing care services, homemaker or home health aide services or personal care services are provided in the homes of the individuals.

In 42 U.S.C.A. § 1397cc(c)(1)(B) categories of basic services described includes "Physicians' surgical and medical services." Thus, the primary, if not universal, definition of "medical services" found in the United States Code is consistent with the Department of Education's definition of medical services as those required to be provided by a physician in 34 C.F.R. § 300.16(b)(4).

The Petitioner's argument, at page 17 of its Brief, concerning "other agencies" is an argument that really has no bearing on this case. Garret has been providing for his services with his own money and insurance. Medicaid may be an available resource to him at some point. *See Detsel v. Sullivan*, 895 F.2d 58, 65-66 (2nd Cir. 1990). Funding from the State of Iowa is also available to the District. The fact that other sources of funding may be available does not mean schools can avoid their obligation to provide related services. Senator Grassley explained that these provisions requiring interagency agreements clarified "that schools are not the only agencies that should pay for the services special education students need." 143 Cong. Rec. § 4407 (daily ed. May 14, 1997); *see also* 143 Cong. Rec. § 4373 (daily ed. May 13, 1997) (Statement of Sen. Murray) ("This bill holds outside agencies responsible for their share of the health or other costs of serving disabled students, so we can clarify that local schools do not bear all the responsibility for these costs."); *id.* at 4300 (Statement of Sen. Harkin) ("[I]n States that have voluntarily provided interagency supports cost savings to [local education agencies] have been significant."); *Hearings on H.R. 5, The IDEA Improvement Act of 1997: Before the House Subcomm. on Early Childhood, Youth & Families of the Comm. on Economic & Educ. Opps.*, 105th Cong., 1st Sess. 46 (1997) (colloquy between Rep. Miller and Assistant Secretary of the Department of Education Heumann as to the need for coordination of payment for health services).

Accordingly, application of statutory construction principles supports the construction in this case of the term "medical services" to mean those performed by a physician.

E. The Lower Court Cases on Which the District Relies Have Erroneously Interpreted this Court's Decision In *Tatro*.

The lower court cases on which the District relies have simply have read requirements into the law that *Tatro* did not impose.

Detsel v. Bd. of Educ. of Auburn, 637 F. Supp. 1022, 1027 (N.D. N.Y. 1986), *aff'd*, 820 F.2d 587 (2nd Cir.) (per curiam), *cert. denied*, 484 U.S. 981 (1987), concluded that while the disputed services did not actually meet the statutory and regulatory definition of "medical services," because the regulation specifically addresses physician-performed services, the denial of the services "is in keeping with [the regulation's] spirit." The problem with this finding is that it ignores the spirit of the IDEA itself. As this Court in *Tatro* stressed, the reason for mandating the provision of supportive services under the IDEA is to guarantee disabled students an opportunity to gain an education. If granting such an opportunity entails furnishing medically-related services short of requiring a physician, such services are the student's right. The IDEA, its legislative history, and its regulations are void of any suggestion that states are free to decide, on the basis of the cost and effort required, which related services fall within the medical services exclusion. *See generally*, *Macomb County Sch. Dist. v. Joshua S.*, 715 F. Supp. 824 (E.D. Mich. 1989) (following *Tatro* and criticizing *Detsel* and its progeny).

Bevin H. v. Wright, 666 F. Supp. 71, 75 (W.D. Pa. 1987), reached its result by holding that related services are only those that are "within reason" and that the proper determination involves a balancing of factors. Yet, *Bevin H's* reliance on *Dept. of Educ., State of Hawaii v. Katherine D.*, 727 F.2d 808 (9th Cir. 1984) (holding that tracheotomy care is a related service), for this proposition is misplaced. In holding that a school district need only provide services within reason, the Court in *Katherine D.* was responding to the question of whether the IDEA must provide a handicapped student "the best possible education." 727 F.2d at 813. This inquiry is entirely separate from the inquiry concerning

the services necessary to provide a child "with meaningful access to education . . ." *Tatro*, 468 U.S. at 891. The issue of the level of education required under the IDEA may raise questions of balance. Yet, once a District determines the appropriate education, the issue of access must be examined within the framework supplied in *Tatro*.

In *Neely v. Rutherford Co. Schools*, 68 F.3d 965 (6th Cir. 1995), the Court of Appeals found it appropriate to take into account the risk involved and the liability factor of the school district. *Id.* at 971. In the case at bar, the District has functionally disavowed any liability concerns by providing these services to other children in the District. At trial it specifically disclaimed any concern that it might be sued as a result of employing a nonlicensed provider. Tr. at 584-85. Further, in *Tatro*, the Court held that the liability risks to school personnel providing the needed service had "no relation to" whether the service was a related service under the IDEA. 468 U.S. at 984 n.12 ("Congress assumed that states receiving the generous grants under the Act were up to the job of managing these new risks"). *Neely* gave no reason for failing to accord any deference to the Secretary of Education's interpretation or failing to follow *Tatro* on the "risk" issue.

Clovis Unified School District v. California Office of Administrative Hearings, 903 F.2d 635 (9th Cir. 1990) (per curiam), likewise gave no reason for failing to apply the Secretary's "related services" regulation and did not acknowledge the *Tatro* Court's reliance on the regulation. *Clovis* involved a psychiatric hospitalization for an emotionally disturbed child. *Id.* at 639. The dispute centered on whether the District was responsible for an admission to an acute psychiatric hospital, at the cost of $150,000 per year, or whether placement in a residential school at the cost of $50,000 was appropriate. *Id.* The District contended that the IDEA did not require it to pay the costs of a psychiatric hospitalization because it was in response to a medical problem rather than an educational need and was thus not the type of residential program contemplated under the IDEA. *Id.* at 642. The Court drew an

analogy between psychiatric hospitalization and two "medical" situations under the IDEA:

> (1) if a child requires, for example, ear surgery to improve his hearing, he may learn better after a successful operation and therefore in some respects his surgery is "supportive" of his education, but the school district is certainly not responsible for his treatment. Similarly (2) a child who must be maintained on kidney dialysis certainly cannot benefit from education to the extent that such services are necessary to keep him alive, but again, it is not the responsibility of the school district to provide such maintenance care.

903 F.2d at 643-44. The Court in *Clovis*, in denying the services, noted that "by law, a licensed physician, though not necessarily the primary treating physician was required to supervise Michelle's overall treatment program." However, the Court in *Tatro* had expressly rejected the District's argument that services provided in accordance with a physician's prescription and general supervision fell within the medical treatment exclusion. It noted that nurses in the school were authorized to dispense medications and administer emergency injections to nondisabled students in accordance with a physician's prescription. 468 U.S. at 893.

Further, the *Clovis* Court noted "that the intensity of [the student's] program indicates that the services she received were focused upon treating an underlying medical crisis." It noted that the less expensive residential placement includes educational services, but the more expensive psychiatric hospital "does not provide its patients with educational services" and thus differed substantially from facilities found by other circuits to be residential placements for which school districts are financially responsible. *Id.* at 646. "The absence of educational services provided by [the psychiatric hospital] indicates that the room and board were medically as opposed to educationally related." *Id.* at 647. The facts of *Clovis* cast substantial doubt on whether the Ninth Circuit would rule the services Garret needs are not required "related services" under the IDEA.

Fulginiti v. Roxbury Township Public Schools, 921 F. Supp. 1302 (D. N.J.), *aff'd without published opinion*, 116 F.3d 468 (3rd Cir. 1997), specifically adopted the ruling from *Detsel* and analyzed the "nature of the services" required and cost of providing the services. *Id.* at 1325. The Court found that the $56,000 annual cost amounted to an "undue burden" and found that the parents had not offered any proof of a lesser priced alternative. *Id.*

In contrast to the cases relied on by the District, a number of courts have followed *Tatro*. In *Tanya v. Cincinnati Board of Education*, 651 N.E. 2d 1373, 1376 (Ohio Ct. App. 1995), the Ohio Court of Appeals required the services of an attendant to monitor and suction a tracheotomy tube every 20-40 minutes under the IDEA. Tracheotomy care has also been held to be a related service. *Dept. of Education v. Katherine D.*, 727 F.2d 809, 815 (9th Cir. 1983). *See also Special School District No. 6, St. Paul, MN*, 23 IDELR 119, 131 (1994) (noting that both "parties assume that tracheal suctioning must be provided by the District"). Catheterization was found to be a related service in *Tokarcik v. Forest Hill School District*, 665 F.2d 443 (3rd Cir. 1981), *cert denied*, 73 L. Ed. 2d 1383 (1983).

In *Morton Community School District No. 709 v. JM*, 986 F. Supp. 1112 (C.D. Ill. 1997), the Court held that under either *Detsel* and its progeny or the *Tatro* bright-line rule that a "pediatric nurse" or "trained" individual to monitor the student during the day was required under the IDEA as a related service. This case was affirmed by the U.S. Court of Appeals for the Seventh Circuit on July 27, 1998, although the Court declined to adopt either a bright-line or a multifactor test. *Morton Community Sch. Dist. No. 709 v. J.M.*, 1998 WL 420393 (7th Cir. 1998). *Morton* involved a fourteen year-old student who breathed through a tracheotomy tube with the intermittent aide of a portable ventilator system. *Id.* at *1. The Court discussed the Department of Education related services regulations and found "the interpretations that the Department of Education places on its own regulations are entitled, of course, to some measure of judicial deference," even when

expressed informally, in a letter or a brief. *Id.* at *3 (citations omitted).

The Court of Appeals rejected the school district's argument, similar to the argument made by the District in this case "that the only health-related services that are not medical services within the meaning of the Act are services traditionally provided by the school nurse." *Id.* at *3. The Court of Appeals found that

> the fallacy in the school district's reliance on the regulation and the *Tatro* decision is the assumption that because services provided by the school nurse are not deemed medical for purposes of the statute, any service that the school nurse can't provide must be deemed medical.

The Court of Appeals also found it significant that the parents could and did provide the services. *Id.* at *3.

Accordingly, contrary to the District's assertion, the Eighth Circuit's decision in this case is in keeping with several other lower courts' decision interpreting *Tatro*.

F. The District's Cost Argument Under the Spending Clause Fails Because the IDEA Establishes Enforceable Rights so as to Put Recipients of Federal Funds on Notice of Their Obligations.

The District, in Section I(D) of its Brief, argues that

> [i]f Congress had intended that School Districts were to provide for costly private nursing services . . . it would have specifically stated so in its definition of "related services."

Petitioner's Brief at p. 20, relying on the Court's spending clause analysis in *Pennhurst State School and Hospital v. Halderman*, 451 U.S. 1 (1981) and subsequent cases. In *Pennhurst*, the Supreme Court held that the "Bill of Rights" provision of the

Developmentally Disabled Assistance and Bill of Rights Act did not create any substantive rights to "appropriate treatment" in the "least restrictive" environment in favor of mentally retarded individuals. 451 U.S. at 18. The Court in *Pennhurst* held that because statutes enacted under the Spending Clause of the United States Constitution are "much in the nature of a contract," an express condition that creates enforceable obligations must be "unambiguously" stated so as to give recipients of grant money fair notice of the condition. 451 U.S. at 17-18. The Court found that the language of the "Bill of Rights" provisions of the Act was merely "hortatory," 451 U.S. at 24, and accordingly did not put the fund recipients on notice that they would be obligated to "spend state money to fund certain rights as a condition of receiving federal money under the Act. . . ." 451 U.S. at 18.

This is not a case like *Pennhurst*, in which the statutory provision involved was one in which Congress was merely giving a "nudge in the preferred direction" through precatory language. *Pennhurst*, 451 U.S. at 19 (citing with approval *Rosado v. Wyman*, 497 U.S. 397, 413 (1970)). This case is different for two reasons. First, the obligation to provide nursing services is imposed independently under Iowa law; thus this is not a case in which the federal government has compelled the state to make expenditures. Second, in contrast to the statute interpreted in *Pennhurst*, the IDEA establishes enforceable rights. *Honig v. Doe*, 484 U.S. 305, 309-10 (1988).[8] The Court in

8. Furthermore, it can be inferred that Congress enacted IDEA pursuant to its Fourteenth Amendment enforcement powers as well as pursuant to the Spending Clause. The Court in *Smith v. Robinson*, 468 U.S. 992 (1984) held that IDEA precludes claims based on the Equal Protection Clause because Congress intended IDEA to be the exclusive "vehicle for protecting the constitutional right of a handicapped child to a public education." *Id.* at 1010. *See also Honig v. Doe*, 484 U.S. 310 (1988) (holding that the EHA was not a "simple funding statute"). The Courts of Appeals have also understood IDEA to be both Section 5 and Spending Clause legislation. *See David D. v. Dartmouth Sch. Comm.*, 775 F.2d 411, 421-422 (1st Cir. 1985), *cert. denied*, 475 U.S. 1140 (1986); *Crawford v.*

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Honig stated after reviewing the Act's legislative history and the problems that were the impetus for the Act:

> In responding to these problems, Congress did not content itself with passage of a simple funding statute, rather the EHA confers upon disabled students an enforceable substantive right to public education in participating states (citations and footnote omitted) and conditions federal financial assistance upon a state's compliance with the substantive and procedural goals of the Act.

484 U.S. at 310.

The Supreme Court has previously addressed and rejected the School District's *Pennhurst* argument that potentially costly services under the IDEA must be specifically enumerated. *Tatro*, 468 U.S. 883, 891 n.8 (1984). In *Tatro* the Court distinguished *Pennhurst* as follows:

> The obligation to provide special education and related services is expressly phrased as a "conditio[n]" for a state to receive funds under the Act. This refutes petitioner's contention that the Act did not "impos[e] an obligation on the States to spend state money to fund certain rights as a condition of receiving federal moneys" but "spoke merely in precatory terms ."

Id. (citations omitted).

In its analysis in *Tatro*, the Court made it clear that a service did *not* need to be specifically enumerated, contrary to the District's argument, as long as it was related to the express condition, which the Court stated was "the obligation to provide special education and related services." 468 U.S. at 891 n.8. As the Court stated, in

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Pittman, 708 F.2d 1028, 1037-1038 (5th Cir. 1983); *Mitten v. Muscogee County Sch. Dist.*, 877 F.2d 932, 938 (11th Cir. 1989), *cert. denied*, 493 U.S. 1072 (1990); *Counsel v. Dow*, 849 F.2d 731, 737 (2d Cir.), *cert. denied*, 488 U.S. 955 (1988) (so holding and collecting older cases).

spite of *not* being enumerated in the Act, a health-related service which permits a child to remain at school during the day is no less "related to the effort to educate than are services that enable the child to reach, enter or exit the school," which were indeed specifically enumerated services in the Act. 468 U.S. at 891.

The remaining Spending Clause cases relied on by the District for the proposition that a costly service does not need to be provided if it is not specifically enumerated do not support the District's position. In *South Dakota v. Dole*, 483 U.S. 203 (1987), the Supreme Court upheld the conditioning of a state's federal highway funds on adoption of minimum drinking age of 21. *Id.* at 207. The fighting issue in *Dole* was whether the Twenty-First amendment constituted an "independent constitutional bar" to the conditional grant of federal funds. *Id.* at 209. In *New York v. United States*, 505 U.S. 144 (1992), the Supreme Court upheld provisions of the Low-Level Radioactive Waste Policy Act. The petitioner did not contest Congress' exercise of its spending powers. 505 U.S. at 172. In dicta, the Court relied on *Pennhurst* for the proposition that the conditions imposed were "unambiguous." *Id.* In *Gebser v. Lago Vista Independent School Dist.*, 118 S. Ct. 1989 (1998), the Court held that under the Spending Clause, the recipients of Title IX funds (where a private right of action for sexual harassment was implied in the first instance) did not have sufficient notice of the imposition of money damages unless the recipient school district had actual knowledge of the harassment. *Id.* at 1998. Here, as this Court made clear in *Tatro*, there is an express condition under the IDEA giving rise to the obligation to provide education and "related services."

The District also relies on *Commonwealth of Virginia, Department of Education v. Riley*, 106 F.3d 559 (4th Cir. 1997), in which the Fourth Circuit Court of Appeals rejected the United States Department of Education's position that a state was required to make special education services available to students with disabilities who were expelled or suspended where the state did not afford the same right to non-disabled students. 106 F.3d at

562. The Court of Appeals, relying on the Supreme Court's articulation of the purpose of the IDEA in *Board of Education v. Rowley*, 458 U.S. 176 (1982), found that the purpose of the Act, which was to "ensure that disabled students are not denied access to a free public education because of their disabilities," was not served by the requirement. 106 F.3d at 559. Here, the purpose of the statute will be served, which is to ensure that a disabled student is not deprived of access.

That the District's cost-related arguments under *Pennhurst* and Congress' spending powers must be rejected is also supported by the Court's decision in *Florence County School District Four v. Carter*, 510 U.S. 7 (1993), in which the Court: (1) recognized the broad scope of a student's right to an education at the district's cost; and (2) rejected the cost arguments of the district. The Court held that the school district was obligated to reimburse the parents of a learning disabled child for the cost of educating the child at a private school because the individualized education plan (IEP) was inadequate under the IDEA. 510 U.S. at 10. The Court so held even though the private school selected by the parents did not meet the IDEA's definition of free appropriate education and the school was not on the state's list of approved schools. 510 U.S. at 12-15. The Court rejected the school district's claim that "allowing reimbursement for parents such as [the student's] puts an unreasonable burden on financially strapped local educational authorities." 510 U.S. at 15. The Court stated:

> [t]here is no doubt that Congress has imposed a significant financial burden on states and school districts that participate in IDEA. Yet public educational authorities who want to avoid reimbursing parents for the private education of a disabled child can do one of two things: give the child a free appropriate public education in a public setting, or place the child in an appropriate private setting of the State's choice. *This is the IDEA's mandate, and school officials who conform to it need not worry about reimbursement claims.*

510 U.S. at 15 (emphasis added).[9]

An additional refutation of the District's cost argument is found in *Morton Community Unit School District No. 709 v. J.M.*, 1998 WL 420393 (7th Cir. 1998). The related services ordered in this case cost about $20,000 per year. *Id.* at *2. The Court noted that the School District had more than 3,000 students so that the annual cost of the nurse would be less than $7 per student. *Id.* at *4. The Court found that the financial burden on the district for the expense of hiring a nurse to attend plaintiff "would be slight." *Id. A fortiori*, the costs of even an additional R.N. in this case is approximately $1 per student. As stated above, there is no additional cost if an LPN is hired in this case.[10]

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9. Further, the District's cost argument is not particularly persuasive in any event, when there are cases such as *Clevenger v. Oak Ridge School Board*, 744 F.2d 514 (6th Cir. 1984) (approving placement of a child in a residential school at a rate of $88,000 per year because cost considerations are only relevant when choosing between several options, all of which offer an appropriate education) and *Cremeans v. Fairland Local School Dist.*, 633 N.E. 2d 570, 581 (Ohio Ct. App. 1993) (finding 24-hour-a-day extended school year residential placement required even though the cost of the program was approximately $94,000 out of a total District budget for education of students with disabilities of $572,000).

10. Petitioner and its amicus exaggerate the scope of their financial obligations by disregarding the fact that there are only a small number of school-age children who have needs comparable to Garret. All the litigated cases involving health monitoring in a regular classroom involve children who rely on ventilators and/or tracheotomy tubes for breathing. The congressional Office of Technology Assessment (OTA) estimated that in any given year there are between 680 and 2,000 children 21 years old and under who receive ventilator assistance. *See* United States Congress, Office of Technology Assessment, *Technology-Dependent Children: Hospital v. Home Care: A Technical Memorandum* 20-21 (1987). OTA also estimated that between 1,000 and 6,000 additional children under 21 breathe with the assistance of tracheotomy tubes. *See id.* at 22-24.

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For the above-stated reasons, the District's reliance on *Pennhurst* is misplaced, and its cost arguments must be rejected.

G. The Bright-line Rule is Practical and is Consistent with Principles of Administration of Justice.

The importance of the bright-line rule is that it eliminates the costs, delay, and the general state of confusion that would result from the imposition of the multifactor test suggested by Petitioner which would require adjudication of varied factual circumstances through extensive appellate processes. *See Schweiker v. Gray Panthers*, 453 U.S. 34, 48 (1981); *Weinberger v. Salfi*, 422 U.S. 749, 782-85 (1975); *Honig v. Doe*, 484 U.S. 310, 322 (1988) (explaining that administrative and judicial review under the IDEA are often "ponderous"). The bright-line test furthers the efficient and proper use of public funds earmarked for education and should avoid litigation over fact-specific disputes engendered by the multifactor test. *See Skelly v. Brookfield LaGrange Park School District 95*, 968 F. Supp. 385, 394 (N.D. Ill. 1997).

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OTA's numbers overestimate the population relevant to school districts in two respects. First, they cover infants and toddlers (that is, children age 0-2) who do not receive school services and who may be weaned from the ventilator or tracheotomy by the time they are ready to enter school. *See* OTA Report, *supra*, at 15, 29. Second, they include children living in hospitals who receive health services from hospitals not school districts. One survey estimates that there are 570 ventilator-dependent patients under age 18 in hospitals. *See* American Association for Respiratory Care, *A Study of Chronic Ventilator Patients in the Hospital*, 11, 14 (1991). Thus the number of ventilator- and tracheotomy-dependent children who attend school and require ventilator/tracheotomy health services is likely at the lower end of the OTA estimates. A survey of school districts by the Iowa Department of Education in 1989 reported that 5 students were using ventilators and 17 were using tracheotomies. Richard Nelson, *et al.*, *A Survey of Special Health Care Procedures Required by Students in Iowa Schools* 2 (Feb. 1990) (copy lodged with the Court). In any event, that population constitutes the barest fraction of the 5.6 million children currently receiving special education and related services. *See* U.S. Department of Education, *Nineteenth Annual Report to Congress on the Implementation of the Individuals with Disabilities Education Act*, II-23 (1997).

In contrast, the multifactor test is time-consuming and expensive in terms of school staff time and effort to apply the test and formulate the annual IEP required by the Act. The inevitable ensuing litigation would also be time-consuming and expensive. Further, the problem with the multifactor analysis proposed by Petitioner is that it suggests no guidance on which, if any, of the factors are determinative or how to weigh the various factors. Such a multifactor test may also result in differing results from one district to the next within the same state.

The bright-line rule further advances the national policy of mainstreaming or integration. This policy is evidenced by 20 U.S.C. § 1412(5) which provides:

> To the maximum extent appropriate, children with disabilities, including children in public or private institutions or other care facilities, are educated with children who are not disabled, and special classes, separate schooling, or other removal of children with disabilities from the regular educational environment occurs only when the nature or severity of the disability of a child is such that education in regular classes with the use of supplementary aids and services cannot be achieved satisfactorily.

Rather than draining financial resources of the federal and state governments, related services enable disabled children to obtain more than a "trivial educational benefit." This insures that the millions of dollars Congress spends for special education programs are not wasted. *See Polk v. Central Susquehanna Intermediate Unit 16*, 853 F.2d 171, 180 (3rd Cir. 1988); S. Rep. No. 168, 94th Cong., 1st Session 5 (1975) (reprinted in 1975 U.S.C.C. Ann. 1425, 1433).

The Petitioner's proposed multifactor test would be a practical nightmare with school districts tenaciously litigating IEP's (individualized educational plans) against financially overmatched

disabled children with the potential outcome of sending the children to home-based or hospital-based education programs. 20 U.S.C. § 1412(5).

If Garret F. is not allowed in school, his educational option, at this point, would be homebound instruction. Pet. App. at 37a-38a; Tr. at 87-88. Even from a purely educational standpoint, however, homebound teaching has significant limitations. It typically consists of only a few hours per week, with a single teacher expected to cover all the subjects the child is studying. American Academy of Pediatrics, *Children With Health Impairments*, 86 Pediatrics, 636 (1990).

The IDEA also requires that the "free appropriate public education" take place to the maximum extent possible with nondisabled children in the least restrictive environment consistent with the needs of the disabled children. 20 U.S.C § 1412(5).

Thus, the three principal advantages of the bright-line rule are that it: (1) effectuates the expressed intent of Congress and interpretation by the Department of Education; (2) it provides a workable standard for educators, disabled children and their families; and (3) it fosters mainstreaming and integration.

II.

EVEN IF THIS COURT DOES NOT FOLLOW THE BRIGHT LINE TEST, THE DECISION THAT THE DISTRICT IS REQUIRED TO PROVIDE THE SERVICES MUST BE AFFIRMED.

The Supreme Court may affirm on any ground in the record. *United States v. Carignan*, 342 U.S. 36, 38-39 (1951); *Thigpen v. Roberts*, 468 U.S. 27, 29-30 (1984).

A. The ALJ's Decision is Entitled to Deference Under This Court's Analysis in *Rowley*.

The ALJ's decision is correct and must be affirmed on alternative grounds. The ALJ ruled in the first instance that "[u]nder

Tatro and *Joshua S.* . . . the health care needs of Garret F. while at school do not require the direct attention and provision by a physician and do not fall within the physician treatment exclusion of related services," finding that under the IDEA such services must be provided by the district by a "qualified school nurse or other qualified person." The ALJ also found that "even when the other tests and criteria found in *Detsel, Bevin H.*, and *Shannon M.* are used, the health care services necessary for Garret are a related service." Pet. App. at 52a. The ALJ distinguished those cases *on their facts* finding that Garret's "general good health, ability to communicate his needs, intelligence, and other factors" distinguished him from the students in those cases, who were "more 'fragile' in the true sense of the word."[11] Pet. App. at 52a.

The facts that the ALJ relied on to find that Garret must be provided the services under a multifactor test included the fact that "it was obvious that in most situations a conscientious lay person could be, and has been in the past, trained to provide the necessary care." Pet. App. at 52a. Barring a change in condition, "there appears no practical need for Garret to be attended by even a licensed health care professional." *Id.* The ALJ's factual findings are entitled to deference. *Rowley*, 458 U.S. at 206.

As the Court stated in *Rowley*:

> [t]he very importance which Congress has attached to compliance with certain procedures in the preparation of an IEP would be frustrated if a Court were permitted

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11. Garret can communicate his status through verbal speech. Tr. at 463. He is in good health. Tr. at 629. No care for the tracheostomy is done at school. Tr. at 34. The trach tube has never been changed at school and the ventilator has never malfunctioned at school. Tr. at 36. If the ventilator did malfunction, Garret can be ambu-bagged manually for a long period of time. Tr. at 36. At the time of trial it had been over one and a half years since there had been any school episodes of autonomic hyperreflexia. Tr. at 81-82. Neither Garret's blood pressure nor his pulse are taken during the school day. Tr. at 648. Garret has never had a close call or an emergency care situation at school. Tr. at 88.

> simply to set state decisions at nought. The fact that § 1415(e) requires that the reviewing Court 'receive the records of the [state] administrative proceedings' carries with it the implied requirement that due weight shall be given to those proceedings . . .

458 U.S. at 206.

The Iowa Department of Education reached the proper conclusion that the District is required to provide an individual to provide the health care services required by Garret F. during the day. This ruling is consistent with the purpose of the IDEA which, in part, is to ameliorate past discrimination against individuals with disabilities and their exclusion from the public education system. Further, it is consistent with the principle enunciated in *Rowley* that, under the Act, state and local educational agencies, in cooperation with the parents, have the primary responsibility for formulating the education to be accorded to a disabled child and that in the face of such a clear statutory directive, it "seems highly unlikely" that Congress intended the Courts to overturn a state's choice of appropriate educational options in a proceeding, such as this one, pursuant to Section 1415(e)(2). 458 U.S. at 207-08.

B. Even Under the District's Multifactor Test, the ALJ's Decision Must be Affirmed.

Under the District's own proposed test, Petitioner's Brief at p. 34-35, on the present record, the District *must* provide the nursing services requested. The District's proposed test is as follows:

> . . . [C]ourts should consider several factors in determining whether the services are excluded medical services or included school health services. These factors include the nature and complexity of the care, whether the care is continuous or intermittent, whether existing school health personnel may provide the services, the cost of the

> services, and the potential repercussions if the services are not properly performed.

Petitioner's Brief at p. 34-35. In order to prevail, the District would have to persuade this Court to reweigh the evidence, which is inconsistent with the deference accorded state boards of education under the IDEA as interpreted by this Court in *Rowley*, 458 U.S. at 206-07. That evidence is as follows.

1. *Nature and complexity of the care*

As the ALJ found, many of the services required by Garret can be and are provided by nonskilled personnel, including family members and unlicensed personal attendants. The District relies on its own IEP team's finding and the original declaratory ruling of the Nursing Board. The District neglects the fact that it was this plan that the ALJ rejected upon appeal by Garret's mother. The District further neglects the fact that the Board of Nursing has now specifically ruled that an LPN can provide the services.

2. *Whether the care is continuous or intermittent*

Despite the District's assertion to the contrary, the only medical evidence, given by Dr. Reinertson, was that Garret did not require continuous monitoring and assessment. Tr. at 627-28. Someone simply needs to be within "earshot" to respond to his needs. Tr. at 628. According to the District, no child, regardless of physical or mental disability, should be left unattended during the school day. Tr. at 410-12. The care he needs is intermittent and it can and has been provided by a lay person. App. 10-11; 19; Tr. at 68-69; 640-41; 653-55.

3. *Whether existing school health personnel may provide the services*

There has been much said about who can provide the services to Garret F. Curiously, although the District claims that the services must be provided by an RN, it has permitted Garret's family to furnish these services throughout the years first with an eighteen-year-old non-professional aunt and then with an LPN. Further, the

District now has an opinion from the Nursing Board that the services may be performed by an LPN.

4. *The cost of the service*

As argued above, Garret F. strenuously denies that costs should be a factor and asserts that there is no controlling precedent that would permit cost to be taken into account once the threshold question of whether the services requested are "supportive services" within the meaning of the IDEA is answered. However, even if cost is a factor, the actual costs of the services rendered in this case have been exaggerated by the District. Finally, the District has access to additional state money to finance these services. Pet. App. 37a; Tr. at 588.

5. *The potential repercussions*

Even if this were to be a relevant factor, which Garret F. denies, there is no evidence that the potential repercussions if the services were performed improperly would be greater if performed by a school nurse as opposed to the LPN hired by the parents. Further, the record in this case is that emergency breathing can be provided for a lengthy period of time through the use of ambu-bagging until emergency medical care arrives. Tr. at 473-74. For example, the Frey family sustained Garret's breath by use of the ambu-bag on a trip from Cedar Rapids, Iowa, to Denver, Colorado, and back. Tr. at 36. Also, the history of Garret at school shows that there has never been an emergency situation. Tr. at 88.

Thus, the District's argument under a multifactor analysis must fail and even if this approach were adopted, the judgment must be affirmed.

III.

AS A FURTHER ALTERNATIVE GROUND, THE JUDGMENT MUST BE AFFIRMED ON STATE LAW GROUNDS.

This Court as a general rule will not affirm on state law grounds. *City of Mesquite v. Aladdin's Castle, Inc.*, 455 U.S. 283, 292 (1982). However, in IDEA cases, state law provisions for special education that create a higher duty for school districts than recognized by federal law have been held to be automatically incorporated into, and enforceable under, the IDEA. *See David D. v. Dartmouth Sch. Comm.*, 775 F.2d at 420. Thus, affirmance is proper under state law grounds. In the alternative, the Court could remand for a determination on state law grounds. *City of Mesquite*, 455 U.S. at 292.

The ALJ held that Garret F. was entitled to the services as "support services" under Iowa law. The Department of Education held that the District had the responsibility under Iowa Code Sections 256B.2 and .3 and Iowa Code Section 280.8 to provide the services. It stated:

> Under [the] Iowa statute Garret is a child requiring special education, and a six-year history of successful functioning in the regular classroom, with aids and services provided by his family and the school prove beyond a doubt that [education in regular classes] can be accomplished satisfactorily. Under state law, Garret must be provided an education program "within the regular school environment" and under the same state law; it is the District's responsibility to do so.

Pet. App. 55a.

The ALJ found that the alternative of a homebound program would "certainly not be an education with nondisabled children to the 'maximum extent appropriate' " within the meaning of federal regulations. Pet. App. 59a. The ALJ also properly noted that under

Iowa law, "only when a child 'cannot sufficiently profit from the work in the regular classroom' may he be removed." Pet. App. 59a; Iowa Code § 256B.8 (1993). The ALJ further found that the Iowa Board of Nursing's declaratory ruling that an RN was required to render the care did not relieve the District of responsibility to provide health care services to Garret at school. Pet App. 60a. Accordingly, the Court should affirm on this ground.

CONCLUSION

The judgment of the Eighth Circuit Court of Appeals should be affirmed.

Respectfully submitted,

DOUGLAS R. OELSCHLAEGER
Counsel of Record
DIANE KUTZKO
SHUTTLEWORTH & INGERSOLL, P.C.
Attorneys for Respondent
500 Firstar Bank Building
P.O. Box 2107
Cedar Rapids, Iowa 52406
(319) 365-9461